

more. It took years before his illness was ultimately diagnosed as OCD. A grueling time for the family, as doctors shillyshallied: one blamed his father for travelling too much, another suspected child abuse. Finally, a psychologist gave his problem a name and the family some hope.

Not all OCD stories have such a happy resolution. But their saga proves both how far medical science has come in understanding and treating the disorder, and how much remains to be done. Research shows that moderate amounts of anxiety can help people perform tasks more efficiently and provide a sense of accomplishment. "And we all have anxious thoughts in some form or the other," says Khanna. But that doesn't mean every person with eccentric traits—say, the woman in office who keeps her desk impeccably neat and gets edgy if something is moved—has OCD. The key to diagnosing whether such behaviour is authentic OCD is, how great an impact the behaviour has on our life. "You have to show longstanding interference with function, and that eliminates most cases," he adds.

The question is: Why do some people get over-active internal alarms? The answer lies in a small structure in the brain called the amygdala, where danger is processed. "If this risk centre is overactive, it would keep on alerting you to peril even after you have attended to the problem," points out Sagar. Functional magnetic resonance imaging (fMRI) show that apart from amygdala, there are other anatomical hot spots involved in the disorder: the orbitofrontal cortex, striatum, anterior cingulum and thalamus. Together, this wiring regulates your response to the stimuli around you, including how anxious you are in the face of threatening or frustrating things. "If there is some imbalance in this circuit, you would fail to inhibit unwanted behaviours," says Reddy.

High-tech scans may

40% patients recover completely, 30% show good improvement, but take 3-4 months to show results

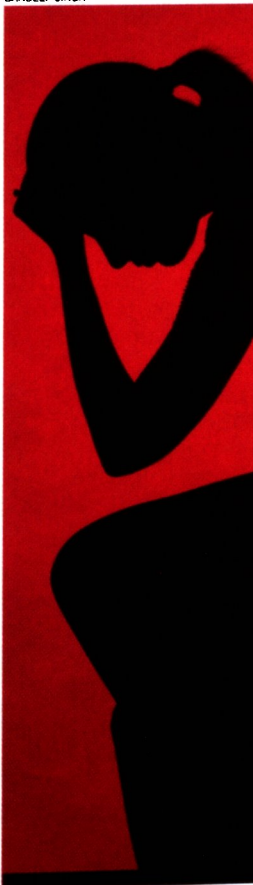
show the landscape of the OCD-torn brain. But they still can't explain what causes the imbalance. "The circuit is a loop, like a string of Diwali lights," says Sagar. "When one goes out, they all go out, so you can't tell exactly where the problem originates." Stress can trigger the disorder, as can strokes. New research shows common streptococcus bacterial infections in children can also be a trigger. Significant is the role of genetics. "About 10 per cent patients report a family member with related disorder," says Reddy.

It wasn't until the symptoms surfaced at age of five that young Vanita's mother linked up her tendency to lick everything—floor, electrical outlets, desks at school—to her husband's idiosyncratic bent to hoard things. "He just can't let go of things—old papers,

tools, books." Vanita was put on a course of anti-depressants, known as selective serotonin reuptake inhibitors (SSRI), a tool doctors now use to control the symptoms. She was also given sessions of cognitive behaviour therapy, where sufferers don't try to avoid a source of anxiety but actually seek it out, until emotional nerve endings grow desensitised. In less than a year, Vanita was much improved, getting As and Bs in school and taking dance classes. Now 11, she goes back to the clinic for occasional booster sessions, and she still takes the anti-depressant. "SSRI and behaviour therapy are effective in 50-60 per cent patients," says Reddy. Researchers are working on the possibility of using surgery and a technique called deep brain stimulation (applying low doses of current) to calm the OCD-tossed brain. Vanita's father, however, is yet to acknowledge his condition and seek help. "He's not willing to talk about it. He still sees it as a stigma," her mother says.

It has been a long march to freedom. For centuries, OCD has been treated as a demonic possession. Even 15 years ago, OCD meant a life-time of agony without relief. Today it has come out of the closet. Yes, football hero, David Beckham, lines up Pepsi cans in the fridge in a certain manner; Hollywood star, Cameron Diaz opens doors with her elbows for fear of contamination; and Michael Jackson goes through surgeries to look better. Demystified in the new millennium, OCD stands as a condition where the brain tries to do something it's wired to do—warning you of danger—but goes overboard in the process. With better treatments breaking its hold, it's time to live without fear. ■

BANDEEP SINGH



CURE CONUNDRUM

Routinely mislabelled as other disorders—depression to schizophrenia

Underdiagnosed and undertreated due to stigma and lack of awareness

OCD striking a young person can mean an entire childhood lost

The disease often leads to breakdown of family life and causes job-related problems

RETHINKING REMEDIES

New research has revealed genetic link

Modern scans can show up the landscape of OCD

New treatments with anti-depressants (selective serotonin reuptake inhibitors) are in force

Cognitive behaviour talk therapy is proving effective